

help requests, nurses couldn't determine the patient who needed immediate help. With voice-based digital assistants, the firm and hospitals anticipated that, with more context from the command text, they could prioritise nurse visits.

The system was deployed with prioritisation based on the words uttered by the patients. For example, if a patient asked for drinking water, the request was flagged as a low priority, whereas if someone complained about pain in the chest, they were flagged with the highest priority. Various other utterances were prioritised accordingly. The idea behind these assigned priorities was that a patient needing water to drink can wait for a few minutes, whereas a patient with chest pain may be at risk of attack and needs immediate assistance. Generally speaking, this logic would work in most of the scenarios.

However, this system wasn't linked with the patient information database. Such that, besides getting the room number of the requester, the system did not know anything else. Most importantly, it did not understand what the patient's condition was and why they were in the hospital.

Not knowing the patient's condition or ailment may prove to be a severe impediment in some cases. These cases may be at the long tail of the common scenarios, and that is what makes it more dangerous.

For example, if a patient has requested water, the system would put it as a low priority request and give other requests a relatively higher priority. However, what if the patient was not feeling well, and hence requested water. May be not getting water in time could worsen the patient's condition. It may be that the patient was choking or coughing and therefore asked for water. Continuous coughing may be an indicator of imminent danger. Without knowing the patient's ailment and condition, it is tough to determine whether a simple request for water



is a high priority or low priority, and these types of scenarios are where I see significant risks. One may term this system as well-intentioned but poorly implemented.

Now the question is—in a scenario where a high priority request gets flagged as a low priority on account of the system's limited information access, who is responsible? If the patient's condition worsens due to this lapse, who would they hold accountable?

Who (or what) can be held accountable for such failures of service or performance is a major lurking question, and perhaps, AI insurance may be able to cover the end-users in all such scenarios when the need for compensation arises.

Many regulators are swiftly gearing up

Since 2018, European lawmakers, legal experts, and manufacturers have been locked in a high-stake debate: whether it's the machines or human beings who should bear ultimate responsibility for their actions.

This debate refers to a paragraph of text that was buried deep in a European Parliament report from early 2017. It suggests that self-learning robots could be granted 'electronic personalities.' This status could allow robots to be insured individually and be held liable for damages, if they go rogue and start hurting people or damaging property.

One of the paragraphs says, "The European Parliament calls on the commission, when carrying out an impact assessment of its future

legislative instrument, to explore, analyse and consider the implications of all possible legal solutions, such as establishing a compulsory insurance scheme where relevant and necessary for specific categories of robots whereby, similarly to what already happens with cars, producers, or owners of robots would be required to take out insurance cover for the damage potentially caused by their robots."

Challenges for AI insurance

Although the market is tilting quickly to justify AI insurance products, there are still a few business challenges. These challenges are clear roadblocks for any one player to take the initiative and lead the way.

A common pool: As the insurance fundamentally works with a common pool, the first challenge appears to be not having enough quorum to start this pool. Several AI solutions companies or adopters can come together and break this barrier by contributing to the common pool.

Equitability: The second challenge is whether this common pool is equitable enough. Due to the nature of AI solutions and customer base, not every company would have equivalent revenue and pose a comparable risk. Being equitable, though may not be mandatory in the beginning, but will soon become an impediment for growth if not appropriately managed.

Insurability of cloud-based AI: In the case of minors (children), parents are liable and responsible for their behaviours in the public domain. Any wrongdoing by them results in parents paying for it. However, as they grow and are termed as adults, responsibility entirely shifts to them.

Similarly, the liability of AI going wrong will have to shift from vendors to users over time, which may impact the annual assessment of premiums. Any updates to the AI software may bring this a few steps backward for vendors as there are new inputs to AI